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Incarnational Psychotherapy: Christ as the Ground of Being for Integrating Psychology and Theology

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Abstract
The integration of psychology and theology is often sought as a layering of intellectual abstractions from biblical and psychological sources. Though scriptural and scientific data may partially define the reality of God and human psychology, the Truth is an embodied act, God made incarnate among us as Christ. The common ground for integration of psychology and theology must then be this ground of being, the incarnate presence of Jesus Christ among us. The basis for integration is derived theologically from the relational nature of and God and the ongoing revelation of Christ to humankind through incarnational presence (i.e., social theology) and psychologically from the preeminence of relationships in shaping human development, well-being, and psychological distress (i.e., interpersonal psychology). This paper reviews both traditions in turn before then discussing the incarnation of Christ as the ground of being for their integration. Incarnational relational enactment is proposed to characterize the transformative presence of Christ in the healing work of a therapeutic relationship. This process is described as immediate, personal, unconditional, and dialectical, ultimately leading to a literal renewing of the mind.

Keywords Incarnational relational enactment · Psychotherapy process · Integration · Social theology · Interpersonal psychology · Corrective emotional experience

This article provides an incarnational model of psychotherapy, integrating social theology with interpersonal psychology. It is our thesis that while biblical and psychological truths provide context for the integration of psychology and Christian theology, only the incarnation of Christ fully reflects the content of integration. The incarnation of Christ is aroused through the dialogical therapy relationship, one exemplar of the kenotic community, where Christ is made manifest in the in-between of the client-therapist relationship. The client and therapist are ushered into the presence of God, and the relationship takes on qualities that reflect the zeitgeist of Christ: immediate, personal, accepting, and dialectical. This incarnational relational enactment leads to a corrective emotional experience (both punctuated experiences and ongoing process) for the client, which in turn leads to neuro- and

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synaptogenesis, going beyond cognitive, behavioral, and affective change to a true biopsychosocial/spiritual transformation for the client.

Stevenson et al. (2007) define the integration of psychology and Christianity as "the task of searching for the unity of truth, seeing things whole from a Christian perspective, and holistically attempting to relate elements of an objective real world given by God at creation" (p. 5). Integration, from their perspective, is the search for a renewed sense of truth through a unity of truth that emerges from combining the truths gleaned from biblical and revelational sources with the truths discovered in psychological and relational sources. The resultant phenomenological understanding is necessarily greater than the sum of its constituent psychological and theological parts.

There can, however, be a problem with a focus on unity of truth if that unity is sought as a layering of intellectual abstractions viewed apart from the ongoing, relational revelation of Christ. Abstract biblical or psychological assertions become trivialized as fixed datapoints and only acquire meaning through a detached process of intellectual interpretation. Truth is considered only as the sum of these constituent data parts, without an experiential understanding of the gestalt through which they are integrated. Truth apart from the ongoing revelation of Christ reduces integration to a focus on abstract principles, and truths become little more than a sandwiching together of contemporary understandings of biblical and psychological concepts, data, and principles—more a parallel process than the discovery of common ground.

Benner's (2007) understanding of integration as incarnational metaphor and Farnsworth's (1985) idea of living as Jesus lived move us closer to an understanding of a Christocentric approach to integration, but we need to recognize that "Jesus did not become incarnate by embodying truths of an abstract or intellectual form. Rather his very being is the truth" (Anderson 1990, p. 187). While the *context* for truth might be biblical revelation and psychological principles, data, etc., the *content* of truth is bound up in Jesus Christ as very man and very God. Tillich (1952) asserted that the essence of Christ cannot be attributed to the model of behavior that he exemplified, his recorded teachings, or the written gospels in which these appear. Instead, Tillich suggested that the essence of Christ is his very being, the relational enactment of Christ among humanity. It is from this emphasis on the content of being that we read Jesus' statement that "I am the way and the truth and the light" (John 14:6 NRSV). Though there are scriptural and historical truths that may partially define the reality of God, the Truth is an embodied act, God made incarnate among us as Christ. The common ground for the integration of psychology and theology must then be this ground of being, the incarnate presence of Jesus Christ among us, and the ongoing revelation that we experience in relation to him.

To fully articulate the centrality of Christ's incarnate being to psychological-theological integration, we will draw on both social theology and interpersonal psychology. Social theology is premised on the relational nature of the triune God and argues for the ongoing revelation of Christ to humankind through his incarnational presence. Interpersonal psychology focuses on the central role of relationships in shaping human development, well-being, and psychological distress. We will begin by reviewing each of these traditions in turn before discussing the incarnation of Christ as the ground of being for their integration. In the final section, we will discuss the incarnational presence of Christ in the therapeutic relationship between client and therapist. We will outline a process of incarnational relational enactment through which the incarnate presence of Christ facilitates true and healing renewal of the mind.

Social theology and the kenotic community

Relational God

God's triune nature is a defining feature of Christian theology. The Trinity simultaneously identifies God as unified (*homoousia)* and differentiated (*exousia*), a single divinity represented in three persons. The most influential work in Western theology to deal with the Trinity, Augustine's *De Trinitate*, attempts to reconcile the inherent tension between God's singularity and multiplicity (Ables 2013). Augustine employed the use of metaphor to illustrate the Trinity, describing the three persons as love, lover, and loved, with the defining feature being the relationship between persons (MacCulloch 2010). Father, Son, and Spirit are constituted by virtue of the relational actions that link them. That is, Augustine defined the Father and Son by their mutual love for one another and the Spirit as the procession of that love from one to the other. Augustine's view of the Trinity is thus unified in and through loving relationship (Ables 2013).

Subsequent readings of Augustine's treatment of the Trinity have focused largely on its relation to the 4th- and fifth century orthodoxy being codified in the language of the Nicene and Apostles' Creeds (MacCulloch 2010). Particular emphasis has been placed on Augustine's contributions to hierarchical understandings of the relationship between Trinitarian elements. As Johnson (1992) has noted, however, the 'procession' of relationships described in Scripture is varied and nonhierarchical. In some passages, it is the Spirit proceeding forth from the Father, in some it is the Spirit giving life to the Son, and in still others it is through the Son that we come to know the Father. Johnson (1992) suggested that Augustine's Trinity be understood primarily—by virtue of its relational language—as an articulation of relationship characterized by mutual giving forth and receiving from. The simultaneous and unified nature of Augustine's Trinity also points to forms of relating that do not involve dominance or hierarchy but are instead premised on mutuality.

Barth (1961) brought about a renewed interest in the theological significance of the Trinity. Arguing for a strict focus on revelation as the primary means of knowing God, Barth articulated his own analogy for the Trinity as revealer, revelation, and revealedness (Torrance 2000). Barth unambiguously stated that God (i.e., creator), God's act of revelation (i.e., Spirit), and the content of that revelation (i.e., Christ) are all unified (1961). As with Augustine, Barth's analogy hinges upon the relational activity between elements of the Trinity. His emphasis on revelation also inherently incorporates human subjectivity into the relational nature of the Trinity. Not only is the Trinity defined by the simultaneous relationality among its elements, but humanity is also entwined in that relationship as the recipient of what is revealed. We are made to be simultaneous actors in the ongoing relational drama of the Trinity.

Barth's theology points to revelation as the means through which humanity is drawn into the relational nature of the divine. The embodied personhood of Christ is considered the defining act of revelation and thus constitutive of God's relational invitation to humanity (Barth 1961). Through the incarnation of Christ, the divine love shared among the persons of the Trinity is extended to humanity. This love is also made vividly, tangibly present among us through an act of fundamental humility. God's self-emptying of divine power and authority in order to take on the human form of Christ is referred to as *kenosis* (Anderson 1979). For Barth, the kenosis is the vessel through which God's relational love is revealed among humankind. Through the kenosis—and thus the incarnation of Christ—humanity is given a new model for relating, an invitation into reconciliation with God and

with one another. We are called to enact the divine love by relating to one another as the incarnated Christ related to us (Romans 14:5). Through the kenosis and incarnation, God teaches us to meet one another as God has met us in Christ.

Kenotic community

Drawing on the kenosis as the defining feature of God's relationship to humanity, the fellowship summoned through incarnation can be understood as one of *kenotic community (*Anderson 1979). The blueprint for this community is provided in and through the personhood of Christ. Its central precept is the self-emptying, empowering love of the kenosis. That same love is repeatedly punctuated in the life and ministry of Christ, and the gospels are replete with archetypal models for kenotic relating. There are the moments in which Jesus subverts relational schema premised on dominance and subservience in order to embrace (Matthew 8:3), invite (Matthew 9:9; Luke 5:27), engage (Luke 19:5–9), and forgive (John 8:11). His rejection of hierarchy echoes the Trinitarian mutuality described by Johnson (1992). Jesus also evokes the presence of this kenotic love as it is enacted among humanity. In Luke 10:33–34, Jesus tells of the good Samaritan being so moved by love that he set down his own preoccupations in order to care for the suffering of another. In Matthew 25:35–40, Jesus affirms acts of hospitality, compassion, and humble relating from one person to another as a fulfillment of the kenotic invitation. These examples all point to God relating to the divine self through humanity. That is, acts of love derived from God reflect back upon God and denote the divine nature (e.g., "God is love," 1 John 4:8; Anderson 1979). The basic calling of the kenotic community is thus to encounter one another through the self-humbling love made known to us in the revelation of Christ. By our participation in the divine life, we become vessels for the transmission of God's love between self and others and between God and humanity.

In the kenosis, Christ divests himself of godliness in order to become human and relate to humanity on human terms. In like manner, the kenotic community divests itself of its human identification in order to participate in the very life of Christ. In doing so, the kenotic community finds true purpose and meaning. Just as the loving relationship between the figures of the Trinity is characterized by mutuality and parity (Johnson 1992), so too are the relationships called forth by Christ in the kenotic community. Those in fellowship must be willing to engage in a self-emptying with regard to the pretenses of dominance and superiority. They must instead allow the spirit of Christ to penetrate their heart with a yielding acceptance that then ensures that they will identify with the other on the same level and footing.

Such humble openness aspires toward genuine relating between persons. It constitutes what Buber (1958) has famously described as I-Thou relating, a level of deep openness to the unfolding of relationship between individuals. Mismeeting, by contrast, includes some level of insecurity, defensiveness, distancing, and claims premised on a perceived hierarchy. Mismeeting constitutes what Buber identified as I-It relating, a view of the other as object rather than as mutually constituted subject.

Both Buber's I-Thou relating and the genuine relating of kenotic community point to the liminal space between mutually constituted subjects as the entry point through which we encounter the divine. God, as the eternal Thou, is apprehended in and through relationship, and the I of each individual is formed in the womblike in-between of I-Thou dialogue. The dialogue allows each member to grow into the fullest embodiment of their own self and to simultaneously embrace the fullest self of the other. This is made possible in and

through an openness to the loving presence of Christ. It is the literal enactment of Jesus' message from Matthew 18:20 that "where two or three are gathered in my name, there am I in their midst" (NRSV). Entering into the genuine relating of the kenotic community thus becomes a vivid experience through which we encounter the incarnate presence of Christ and, through Christ's spirit, participate in the divine life.

Interpersonal psychology

Interpersonal neurobiology

Human development is a complex, recursive system of interrelationships among the biological, psychological, and social aspects of personhood. We are formed, nurtured, and socialized in and through relationship. Psychology has made significant strides in the past 20 years toward formally recognizing the significance of relationship in the formation of personality and development of brain neurology (Schore 2011). The interdisciplinary field of interpersonal neurobiology has emerged to account for the role of caregiver relationships in childhood brain development (Gambrel et al. 2016). The core premise of interpersonal neurobiology is that healthy relationships with caregivers in infancy and early childhood allow the brain to develop optimal neural connections between the limbic system and the prefrontal cortex (Fishbane 2007). These neural connections are then thought to facilitate adaptive psychological processes such as emotion regulation, acceptance, empathy, and coping (Siegel 2007). Relationships with early caregivers (i.e., attachment) are central to this process as they represent the initial interpersonal milieu through which infants and children learn to relate to themselves and their environment (Siegel 2007; Vaughn et al. 2008).

Much of early brain activity occurs in the right orbitofrontal cortex (OFC), the region of the brain most central to attachment and the attachment relationship (Minagawa-Kawai et al. 2009; Parsons et al. 2013). The OFC serves the vital integrating function of coordinating social communication, empathic attunement, emotional regulation, registration of body states, stimulus appraisal, and self-awareness (Siegel and Hartzell 2003). One's genetic blueprint erects a rough neural scaffold of micro and macro anatomy connecting the OFC with the more primal limbic areas such as the amygdala and thalamus (Fishbane 2007). These limbic structures are key for the processing of adverse emotions, particularly fear and distress (LeDoux 2002). Early relational experiences fine-tune these neural connections between the OFC and the limbic system into the specific neural pathways that help to shape a person's emotional identity (LeDoux 1998). It is generally believed that relational experiences of acceptance, nurturance, and affection in infancy and early childhood substantially aid in the normative development of these neural connections. Such theoretical assertions are supported by imaging studies associating supportive and attentive parenting with more developed neural circuitry for emotion regulation and stress management (Luby et al. 2012). Relationships thus represent the "ongoing experience [that] gradually transforms a child's neural configuration... one synapse at a time" (Lewis et al. 2000, p. 151).

Relationships continue to play an important role in brain development throughout the lifespan. The presence of supportive caregiver relationships in childhood has been shown to predict emotional stability and self-acceptance in adolescence as well as the quality of romantic relationships in adulthood (Gambrel et al. 2016). During the transition to

parenthood, changes in brain anatomy take place as parents prepare for and then form new relationships with the infant and child (Kim et al. 2010; Lambert 2012). Long-term friendships and romantic relationships have also been associated with both brain structure and function in late adulthood (Acevedo et al. 2012). These examples serve as a testament to the importance of interpersonal connections over time in shaping normal psychological functioning.

Psychopathology as broken relationship

When viewed from the perspective of interpersonal psychology, psychopathology represents a rupture in the relationships that help constitute healthy functioning (Thoburn and Sexton 2016). Given the importance of secure relationships in cultivating emotion regulation, empathy, and self-awareness, ruptures in our core relationships are thought to have a detrimental effect on normal functioning (Slipp 1984). All relationships can have an impact on psychological functioning, but close and emotionally intimate relationships are particularly important. So too is the relationship that we have with ourselves—comprised of the myriad of thoughts, beliefs, and feelings that we have about the self. When either form of core relationship—either our relationship with the self or our relationship to those with whom we have been emotionally vulnerable—are ruptured, there is a resulting dysregulation of our psychological systems (Siegel 2007; Slipp 1984).

This has been most clearly demonstrated through the research on various forms of insecure attachment. Disruptions in the infant-caregiver relationship as a result of neglect, inconsistency, or hostility can lead to poor emotion regulation capacities on the part of the infant (Zimmer-Gembeck et al. 2017). From a cognitive perspective, these insecure attachment relationships can also lead to the development of foundational schemas for viewing the self as damaged or inadequate and other people as dangerous or untrustworthy (Simard et al. 2011). Maladaptive schemas formed through disrupted attachment relationships in infancy can have a subsequently detrimental effect on other interpersonal relationships later in life (McLean et al. 2014). The brain is hardwired for survival, and these maladaptive schemas frequently employ an underlying fear response that is mediated by the limbic system (Fellous and LeDoux 2005). This conditioned fear response results in aversion to the healthy forms of emotional intimacy that characterize adaptive interpersonal functioning.

One of the hallmark features of a psychiatric disorder as defined by the most recent diagnostic manual is impairment in one's interpersonal relationships (American Psychiatric Association 2013). While a simplistic causal attribution might view such impairment as resulting from the disorder itself, a wealth of contemporary research points to the bidirectional relationship between intrapersonal psychological distress and impaired interpersonal relationships (Whisman and Beach 2013; Whisman and Uebelacker 2003). Discord in interpersonal relationships can cause, trigger, or compound psychological distress. That same psychological stress can also make it difficult for individuals to fully engage in the behaviors that constitute a healthy interpersonal relationship.

Limbic revision

Just as relationships can be a source of psychological distress, so too can they be a source of much-needed healing. At a neurological level, the healing that occurs within relationship likely unfolds through a process that again involves the linkages between the limbic and the prefrontal cortex structures of the brain. This neurological change occurs when

novel experiences are strong enough to aid in the formation of new neurons and synaptic pathways between these structures. The limbic system is thought to be of primary importance in the processing of emotions, particularly the fear response that characterizes core maladaptive schemas (LeDoux 1998). Repeated exposure to emotionally salient social interactions in the absence of an aversive stimulus allows for the deconditioning of that same fear response (Hayes et al. 2008). This process is likely aided by the genesis of new neurons in the hippocampus region of the limbic system (i.e., neurogenesis; Apple et al. 2017), the strengthening of synaptic connections associated with adaptive emotion regulation (i.e. synaptogenesis; Aupperle et al. 2013; Cisler et al. 2014, 2016), and the deterioration of synapses involved in dysregulation (Li et al. 2009). Therapeutic change may thus rely on the complex processes of neurogenesis and synaptogenesis that occur in the context of relationship.

These findings lend support to the Lewis et al. (2000) view that one person can aid in restructuring the limbic brain of another, which they call *limbic revision*. This claim can be summarized by their axiom that neural patterns established through interpersonal relationship can only be revised through the milieu of interpersonal relating. The work of psychotherapy is, at its core, a process of interpersonal limbic revision. Buber suggested that learning and formation occur in the in-between of persons, that is, where persons intersect through dialogue in all its forms (Kramer and Gawlick 2003). An I-It relationship is characterized by one's view of the other as object and as fixed in time and there to be acted upon. To use the language of psychology, it is to see the other only through the lens of our own preexisting cognitive schemas and the expectation that patterns experienced in the past will be fulfilled in the present. Buber's characterization of the I-Thou relationship, in contrast, is premised on mutual subjectivity in which both individuals are dynamic actors that bring life to the dialogue between them. I-Thou relating creates a fertile holding place in which the processes of learning and formation take place. The actors move beyond expectations based on preestablished schemas and instead see one another in the fullness of their mutual humanity.

This space between I and thou is the fomenting source of limbic revision and healing. When individuals experience one another as dynamic actors, fully embracing the humanness of the other, they are freed to relate in new ways. They are no longer tethered to previous dysfunctional patterns or schemata; instead, they are able to respond to one another in each moment with genuine love and affirmation. This gives rise to a series of punctuated, emotionally salient interactions that form the basis for limbic revision. These corrective emotional experiences occur when thoughts, feelings, and behaviors that previously perpetuated broken relationships are instead responded to with thoughts, feelings, and behaviors rooted in love (Teyber and McClure 2011). Over time, experiential stimuli previously associated with fear or hurt—and thus a need for corresponding self-protection—become neurologically linked with greater self-regulation (Tabibnia and Radecki 2018). Most central to the development of healthy interpersonal relating is the process through which individuals become increasingly comfortable with the vulnerability necessary for emotional intimacy (Lewis et al. 2000). Through the corrective emotional experiences of genuine I-Thou relating, they learn that vulnerability and intimacy, rather than hurt and harm, are conducive to deeper relationship.

Research on psychotherapy outcomes indicates that it is the therapeutic relationship, much more so than the therapeutic approach or formalized techniques, that gives rise to the change associated with corrective emotional experiences (Norcross 2011). Limbic revision is thus premised on genuine relating rather than the leveraging of specific data or a particular psychological theory.

Integration and the incarnational relational enactment

Efforts to integrate psychology and theology frequently focus on the layering of parallel data. Our review of social theology and interpersonal psychology in this paper suggests that relationship, rather than data, lies at the core of healing. The essence of what it is to be Christian, therefore, is to take part in the kenotic community by embracing the relational presence of Christ. In like manner, the therapeutic relationship is one manifestation of the kenotic community. Participation in the kenotic community of the client-therapist relationship and the I-Thou relating that facilitates corrective emotional experience are both contingent upon a humbling, genuine relationship without pretense of power or hierarchical authority. From a Christian standpoint, this form of relating can be understood most simply as an enactment of love made possible through the presence of Christ.

Recognizing the relational presence of Christ at the core of both psychological and theological healing allows for the use of a new term to describe the corrective emotional experience. These therapeutic encounters only unfold within the real presence of Christ, and the real presence of Christ can occur only within the context of human encounters (Anderson 1990, pp. 186–187). We refer to these punctuated moments of heightened emotional experiencing, fomented in and through the presence of Christ, as *incarnational relational enactment*. Drawing together both social theology and interpersonal psychology, we describe incarnational relational enactment as immediate, personal, premised upon unconditional acceptance, and dialectical.

Immediacy refers to meeting in the here-and-now moment, a meeting that apprehends the full potential for transformation in each moment**.** Immediacy is the gateway to attunement, critical moments in therapy when the therapist is completely in sync emotionally, mentally, and spiritually with the client. It is also contingent upon the theological assertion that revelation is not chronologically fixed but instead occurs in each new moment through the spirit of Christ. This is the relational nature of revelation described by Barth (1961). Through the resurrection, the incarnational presence of Christ transcends the temporal specificity of 1st-century Palestine (Anderson 1979). Christ is present in the immediacy of each new moment of history. The spirit of Christ is thus present in each new moment of therapy, perpetually inviting both therapist and client into greater attunement with one another.

This attuned relating is also highly personal. Therapist and client are immersed in the presence of Christ, who knows each individual down to the numbering of the hairs on their head (Matthew 10:30). The relationship thus takes the form of an encounter in which both participants seek to know and be known by the other**.** To be truly known by another is to be seen not as a collection of expected behaviors or prior schemas but as a free and fulsome individual. This is what Barth (2003) describes as "man's free confrontation with his fellow man" (p. 7) and what Tillich (1952) identifies as the "courage to affirm the self" through encounter with another (p. 91). Understanding the unique personhood that constitutes the other is necessary for accurate empathy and affirmation (Miller and Rollnick 2013) and client response specificity (Teyber and McClure 2011), all of which are instrumental in a corrective emotional experience. Knowing and being known is ultimately vital to a healed existence because, as Anderson (1979) notes, "It offers each person... an actual growth into the reality of his own personhood, which is the capacity to live in love" (p. 313). It is the reestablishment of a positive movement of being in relation to an other that makes therapy therapeutic (Lee and Balswick 1989, p. 306).

Incarnational relational enactment also moves beyond knowing into a state of unconditional acceptance. The very essence of incarnation is God's continual embrace of the flawed human condition. In therapy, this takes the form of each participant being received into relationship by virtue of their common humanity with Christ, without any requisite qualifying characteristics or actions (Anderson 1979). Incarnational relational enactment allows the client to experience affective states that they have traditionally repressed because of fear of punishment, rejection, abandonment, or being used. The great therapeutic act is in allowing the client the paradox of feeling and expressing both love and hate toward the therapist—to initiate feelings of a potentially killing rage, knowing that the therapist will survive (Winnicott 1988). Again, this is not the therapist simply acting like Jesus; rather, it is a yielding to and participation in the healing movement of Christ. The incarnational relational enactment thus allows the client to express the fullness of their personhood without fear of rejection or dismissal, to experience loving relationship without qualifications or conditions.

The unqualified love of Christ is held in perpetual tension with the simultaneous call to repentance. God both accepts and reforms. These distinctive theological theses form two poles of the dialectic in which we encounter the spirit of Christ. Growth and transformation, often described as the process of becoming, constitute the synthesis of these poles. Through the incarnational relational enactment, this dialectic characterizes the therapeutic process. The client is accepted as they are, free to enact their personhood without fear of rejection. At the same time, the therapist draws on the intimacy of I-Thou relating to explicitly identify maladaptive patterns and formulate new, more adaptive responses (Teyber and McClure 2011). When synthesized, this simultaneous acceptance and change allows for therapeutic growth.

In the original Greek of the New Testament, such a transformation of the mind was described as *metanoia*. Often written in modern English as repentance, the word more literally translates as a turning (*meta*) of the mind (*noia*). This metanoia is the telos of the incarnational relational enactment, its ultimate purpose. Metanoia is the summation of the neurogenesis and synaptogenesis that underlie limbic revision, which in turn changes the effortful process of learning new ways of interacting with the self and with others into an effortless process of being in relationship with others. It is a renewal of personhood, the capacity to live in love (Anderson 1979), and a willingness to relate to another—and ultimately to Christ—with intimacy and vulnerability.

Conclusion

Psychological and biblical truths provide context for integration, but the content of integration is the ground of being, Christ himself. Through Christ, the relational nature of the divine is extended to humankind. The love extended by God through Christ is grounded in self-emptying kenosis, and humankind is invited to live into this love through participation in the kenotic community. The therapeutic relationship constitutes one manifestation of the kenotic community, where Christ is made manifest in the midst of the caring relationship. It is through the therapeutic dialogue that both client and therapist encounter God's life-altering love and healing.

The interpersonal process between therapist and client is optimally suited for attaining therapeutic change because it leverages the relational nature of human neurology and psychology. Both adaptive and maladaptive human development are formed in relationship.

Therapeutic healing is thus premised on an improved capacity for relational functioning. The client must undergo iterative corrective emotional *experiences* that allow for a process of limbic revision. Experiential stimuli previously associated with fear and hurt are, through genuine I-Thou relating, gradually paired with experiences of acceptance and self-regulation. New neurons and synaptic connections are formed that ultimately sustain this more adaptive relational framework.

This process of therapeutic change is made possible in and through the presence of Christ and the manifestation of kenotic community. The presence of Christ in the midst of therapy gives rise to moments of incarnational relational enactment. Each participant in the therapeutic dialogue enters a state of heightened connectedness to self, other, and Christ. In this heightened state of connecting, there is a focus on the immediate, here-and-now nature of God's relational revelation in each new moment. The attunement is also deeply personal, allowing the client to grow into the fullness of their personhood. It is premised on unconditional, unqualified acceptance of the other and yet simultaneously calls for a process of growth and change. The resulting dialectic is synthesized through limbic revision, resulting in a literal renewing of the mind and leading to a new capacity to live in love.

References

Ables, T. (2013). *Incarnational realism: Trinity and the Spirit in Augustine and Barth* (Vol. 2). T & T Clark Studies in Systematic Theology. London: Bloomsbury T & T Clark.

Acevedo, B. P., Aron, A., Fisher, H. E., & Brown, L. L. (2012). Neural correlates of long-term intense romantic love. *Social Cognitive and Affective Neuroscience, 7*(2), 145–159. https://doi.org/10.1093/scan/nsq092.

American Psychiatric Association (2013). *Diagnostic and statistical manual of mental disorders* (5th ed.). Washington: Author.

Anderson, R. (1979). *Theological foundations for ministry: Selected readings for a theology of the church in ministry*. Edinburgh: Grand Rapids, MI: T. & T. Clark; Eerdmans.

Anderson, R. S. (1990). *Christians who counsel*. Pasadena: Fuller Seminary Press.

Apple, D. M., Fonseca, R. S., & Kokovay, E. (2017). The role of adult neurogenesis in psychiatric and cognitive disorders. *Brain Research, 1655,* 270–276. https://doi.org/10.1016/j.brainres.2016.01.023.

Aupperle, R. L., Allard, C. B., Simmons, A. N., Flagan, T., Thorp, S. R., Norman, S. B., … & Stein, M. B. (2013). Neural responses during emotional processing before and after cognitive trauma therapy for battered women. *Psychiatry Research: Neuroimaging, 214*(1), 48–55. https://doi.org/10.1016/j.pscychresns.2013.05.001.

Barth, K. (1961). In H. Gollwitzer (Ed), *Karl Barth's church dogmatics* (G. W. Bromiley, Trans.) (pp. 295–333). Edinburgh: T. & T. Clark.

Barth, K. (2003). *God Here and Now*. New York: Taylor & Francis.

Benner, D. G. (2007). The incarnation as a metaphor for psychotherapy. In D. H. Stevenson, B. E. Eck, & P. C. Hill (Eds.), *Psychology and Christianity integration* (pp. 247–252). Batavia: CAPS.

Buber, M. (1958). *I and Thou*. New York: Scribner.

Cisler, J. M., Sigel, B. A., Steele, J. S., Smitherman, S., Vanderzee, K., Pemberton, J., … & Kilts, C. D. (2016). Changes in functional connectivity of the amygdala during cognitive reappraisal predict symptom reduction during trauma-focused cognitive–behavioral therapy among adolescent girls with posttraumatic stress disorder. *Psychological Medicine, 46*(14), 3013–3023. https://doi.org/10.1017/S0033291716001847.

Cisler, J. M., Steele, J. S., Lenow, J. K., Smitherman, S., Everett, B., Messias, E., & Kilts, C. D. (2014). Functional reorganization of neural networks during repeated exposure to the traumatic memory in posttraumatic stress disorder: An exploratory fMRI study. *Journal of Psychiatric Research, 48*(1), 47–55.

Farnsworth, K. (1985). *Wholehearted integration: Harmonizing psychology and Christianity through word and deed*. Peabody: Baker Book House.

Fellous, J., & Ledoux, J. E. (2005). Toward basic principles for emotional processing: What the fearful brain tells the robot. *Who needs emotions? The brain meets the robot* (pp. 79–115). New York: Oxford University Press. https://doi.org/10.1093/acprof:oso/9780195166194.003.0004.

Fishbane, M. D. (2007). Wired to connect: Neuroscience, relationships, and therapy. *Family Process, 46,* 395–412. https://doi.org/10.1111/j.1545-5300,2007.00219.x.

Gambrel, L. E., Faas, C., Kaestle, C. E., & Savla, J. (2016). Interpersonal neurobiology and couple relationship quality: A longitudinal model. *Contemporary Family Therapy: An International Journal, 38*(3), 272–283. https://doi.org/10.1007/s10591-016-9381-y.

Hayes, S. A., Hope, D. A., & Heimberg, R. G. (2008). The pattern of subjective anxiety during in-session exposures over the course of cognitive-behavioral therapy for clients with social anxiety. *Behavior Therapy, 39*(3), 286–299. https://doi.org/10.1016/j.beth.2007.09.001.

Johnson, E. A. (1992). *She who is: The mystery of God in feminist theological discourse*. New York: Crossroad.

Kim, P., Leckman, J. F., Mayes, L. C., Feldman, R., Wang, X., & Swain, J. E. (2010). The plasticity of human maternal brain: Longitudinal changes in brain anatomy during the early postpartum period. *Behavioral Neuroscience, 124*(5), 695–700. https://doi.org/10.1037/a0020884.

Kramer, K. P., & Gawlick, M. (2003). *Martin Buber's I and Thou: Practicing living dialogue*. Mahwah: Paulist Press.

Lambert, K. G. (2012). The parental brain: Transformations and adaptations. *Physiology & Behavior, 107*(5), 792–800. https://doi.org/10.1016/j.physbeh.2012.03.018.

LeDoux, J. (1998). *The emotional brain: The mysterious underpinnings of emotional life* (1st Touchstone). New York: Simon & Schuster.

LeDoux, J. (2002). *Synaptic self: How our brains become who we are*. New York: Viking.

Lee, C., & Balswick, J. (1989). *Life in a glass house: The minister's family in its unique social context*. Grand Rapids: Zondervan.

Lewis, T., Amini, F., & Lannon, R. (2000). *A general theory of love* (1st Vintage). New York: Vintage Books.

Li, G., Nair, S. S., & Quirk, G. J. (2009). A biologically realistic network model of acquisition and extinction of conditioned fear associations in lateral amygdala neurons. *Journal of Neurophysiology, 101*(3), 1629–1646. https://doi.org/10.1152/jn.90765.2008.

Luby, J. L., Barch, D. M., Belden, A., Gaffrey, M. S., Tillman, R., Babb, C., … & Botteron, K. N. (2012). Maternal support in early childhood predicts larger hippocampal volumes at school age. *Proceedings of the National Academy of Sciences, 109*(8), 2854–2859. https://doi.org/10.1073/pnas.1118003109.

MacCulloch, D. (2010). *Christianity: The first three thousand years* (1st American). New York: Viking.

McLean, H. R., Bailey, H. N., & Lumley, M. N. (2014). The secure base script: Associated with early maladaptive schemas related to attachment. *Psychology and Psychotherapy: Theory, Research and Practice, 87*(4), 425–446. https://doi.org/10.1111/papt.12025.

Miller, W., & Rollnick, S. (2013). *Motivational interviewing: Helping people change* (3rd ed.). Applications of Motivational Interviewing. New York: Guilford Press.

Minagawa-Kawai, Y., Matsuoka, S., Dan, I., Naoi, N., Nakamura, K., & Kojima, S. (2009). Prefrontal activation associated with social attachment: Facial-emotion recognition in mothers and infants. *Cerebral Cortex, 19*(2), 284–292. https://doi.org/10.1093/cercor/bhn081.

Norcross, J. C. (2011). Psychotherapy relationships that work: Evidence-based responsiveness (2nd ed.). New York: Oxford University Press. doi:https://doi.org/10.1093/acprof:oso/9780199737208.001.0001.

Parsons, C. E., Stark, E. A., Young, K. S., Stein, A., & Kringelbach, M. L. (2013). Understanding the human parental brain: A critical role of the orbitofrontal cortex. *Social Neuroscience, 8*(6), 525–543. https://doi.org/10.1080/17470919.2013.842610.

Schore, A. N. (2011). *The science of the art of psychotherapy*. New York: W. W. Norton.

Siegel, D. J. (2007). *The mindful brain*. New York: W. W. Norton.

Siegel, D. J., & Hartzell, M. (2003). *Parenting from the inside out*. New York: Penguin.

Simard, V., Moss, E., & Pascuzzo, K. (2011). Early maladaptive schemas and child and adult attachment: A 15-year longitudinal study. *Psychology and Psychotherapy: Theory, Research and Practice, 84*(4), 349–366. https://doi.org/10.1111/j.2044-8341.2010.02009.x.

Slipp, S. (1984). *Object relations: A dynamic bridge between individual and family treatment*. Northvale: J. Aronson.

Stevenson, D., Eck, B. E., & Hill, P. C. (2007). *Psychology and Christianity integration: Seminal works that shaped the movement*. Batavia: Christian Association for Psychological Studies.

Tabibnia, G., & Radecki, D. (2018). Resilience training that can change the brain. *Consulting Psychology Journal: Practice and Research, 70*(1), 59–88. https://doi.org/10.1037/cpb0000110.

Teyber, E., & McClure, F. H. (2011). *Interpersonal process in therapy: An integrative model* (6th ed.). Belmont: Brooks/Cole Cengage Learning.

Thoburn, J., & Sexton, T. L. (2016). *Family psychology: Theory, research, and practice*. Santa Barbara: Praeger.

Tillich, P. (1952). *The courage to be*. New Haven: Yale University Press.

Torrance, A. (2000). The Trinity. In J. Webster (Ed.), *The Cambridge Companion to Karl Barth* (pp. 72–92). New York: Cambridge University Press.

Vaughn, B. E., Bost, K. K., & van IJzendoorn, M. H. (2008). Attachment and temperament: Additive and interactive influences on behavior, affect, and cognition during infancy and childhood. In J. Cassidy & P. R. Shaver (Eds.), *Handbook of attachment: Theory, research, and clinical applications* (pp. 192–216). New York: Guilford Press.

Whisman, M. A., & Beach, S. H. (2013). Relationship distress: Assessment, definition, and implications for mental health diagnosis. In H. M. Foran, S. H. Beach, A. S. Slep, R. E. Heyman, M. Z. Wamboldt, H. M. Foran, . . . & M. Z. Wamboldt (Eds.), *Family problems and family violence: Reliable assessment and the ICD-11* (pp. 71–78). New York: Springer.

Whisman, M. A., & Uebelacker, L. A. (2003). Comorbidity of relationship distress and mental and physical health problems. In D. K. Snyder & M. A. Whisman (Eds.), *Treating difficult couples: Helping clients with coexisting mental and relationship disorders* (pp. 3–26). New York: Guilford Press.

Winnicott, D. W. (1988). *Human nature*. New York: Bruner Mazel.

Zimmer-Gembeck, M. J., Webb, H. J., Pepping, C. A., Swan, K., Merlo, O., Skinner, E. A., … & Dunbar, M. (2017). Review: Is parent–child attachment a correlate of children's emotion regulation and coping? *International Journal of Behavioral Development, 41*(1), 74–93.

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